

Expert Interview Summary — Dr Gary Kui-Kai Lau

EngD Research Record — Expert Consultation Series (second consultation, June 2026)

Project: Leveraging Gamification as a Proactive Approach for Continuous Improvement and Innovation Enhancement

Researcher: Sidney K. T. Tam · Lead Supervisor: Prof. Benny C. F. Cheung

*Named with the participant's written authorisation · DRAFT for author review and participant validation
— prepared 11 August 2026*

1. Participant and standpoint

Dr Gary Kui-Kai Lau is Assistant Dean (Education Innovations) and Clinical Associate Professor in the Division of Neurology, LKS Faculty of Medicine, The University of Hong Kong (ORCID 0000-0002-8657-2418). A stroke neurologist, he founded the HKU STROKE Research Group in 2018 and has led its programme of stroke research and prevention education; in recent years his work includes AI-enabled mobile applications supporting stroke patients' recovery. In his public commentary (including a recent RTHK interview on his values in stroke care and medical education) he describes a mind constantly "setting off fireworks" with new ideas, and holds a stated position directly relevant to this programme: technology should be embraced but not over-relied upon — care must remain people-centred, and the warmth of human interaction cherished.

Relationship disclosure: Dr Lau and the researcher have worked as peers on games supporting his stroke-prevention education, a collaboration that preceded and motivated this consultation. His standpoint on the panel is hospital neurology and rehabilitation practice — the panel's clinical-medicine anchor alongside gerontology (Dr Yuan), physiotherapy (Expert P) and naturopathic practice (Dr Choy).

2. Consent and interview record

Interview: June 2026 (the second of the four consultations), in person, approximately one hour, in Cantonese mixed with English; the researcher's written notes are the primary record. Consent: completed digitally on 10 August 2026 — all nine statements initialled "GL", signature image on the participant line — and countersigned by the researcher on 11 August 2026. Identification: Dr Lau returned the form with neither preference box marked; asked directly, he elected named attribution in writing ("Ok to be identified", WhatsApp, 11 Aug 2026), and the researcher's tick on the form transcribes that written election — both artifacts are archived. Under the programme's dual-dating rule: interview June 2026; written consent 10-11 August 2026, obtained retrospectively.

3. Summary of Dr Lau's views (researcher's record)

3.1 Patience and the long journey. In his clinical experience, patients generally lack the patience to persist with health technology when improvement takes a long time to show — and stroke rehabilitation is a very long journey. Sustained engagement, not initial willingness, is the scarce resource.

3.2 What gamification contributes. Gamification helps patients become involved and participate with joy, which he regards as a very important element. The researcher and Dr Lau have applied this jointly in his stroke-prevention education work.

3.3 The limits of joy. The joy of play faces real limits: the long recovery journey is difficult, boring and lonely, and much current health technology offers no progressive, positive signs to hold onto. He believed gamification may be able to address this limitation of current technologies — the absence of visible progress, as much as the absence of fun.

3.4 Giving up. On dropout when health technology proves difficult: the elderly community gives up easily, even when facing serious health conditions — even though, if they persisted, they would likely be in better shape medically. Many elderly patients' post-surgical rehabilitation outcomes are unsatisfactory for this reason; they surrender their chance of recovery. In stroke rehabilitation specifically, patients generally follow the guideline at first; some give up shortly after; very seldom do they try once and drop out immediately. The tragic pattern is eventual abandonment — staying as they are, never recovering — and it is not a minority group.

3.5 Honest uncertainty. Whether games can prevent this abandonment, he cannot be sure.

3.6 What would work — and why it does not scale. Coaching is, in his view, always the best and most direct approach, especially when family members join the rehabilitation process — but it is not practical: the social cost is far too high, and even professional therapist resources are hard to secure. Gamification can contribute the joy and fun; whether it can change patients' mindsets or behaviours he did not yet know — "but it is a good start to find out how effective gamification can be in rehabilitation health."

4. Relation to the thesis

(a) The coaching argument (§7.5, L6): Dr Lau independently identifies coaching as the most effective intervention AND establishes why human coaching cannot scale (family cost; therapist scarcity). This is the strongest practice-based justification in the panel record for the framework's coaching layer — and for AI's enabling role as the only affordable way to give every patient a coach. (b) Trajectory visibility (§7.5): his diagnosis that current technology lacks 'progressive, positive signs' across a boring, lonely journey maps directly onto the framework's visible-trajectory element, and connects to the loneliness findings of the PRISM literature already cited (§8.6B). (c) Sustained engagement as the scarce resource (§5.5, §8.5): his patience account converges with the programme's retention findings from a clinical population. (d) TIMING NUANCE, recorded honestly: in stroke rehabilitation he sees few first-attempt dropouts — abandonment comes later in the journey — whereas the FRA pattern showed early adverse-first-experience effects. The populations differ (clinically supervised rehabilitation under medical necessity vs voluntary preventive assessment), and the thesis must present his account as an adjacent-population perspective, not direct corroboration of FIDS timing. (e) People-centred technology: his embrace-but-do-not-over-rely position independently states the thesis's V32 boundary — AI as enabling tool, the human mechanism central. CAUTION: as with Expert P, Dr Lau was not asked the verdict-vs-game-failure attribution question; attribution corroboration rests

on Dr Choy alone. Dr Lau's honest uncertainty (3.5, 3.6) is reported as such — his contribution is interpretation and clinical contextualisation, never a substitute for evidence.

5. Records checklist

Archived: signed + countersigned consent form (10-11 Aug 2026); written attribution election (WhatsApp screenshot — to be re-sent as a file); this summary drafted from the author's account of 11 Aug 2026. RECOMMENDED: send this summary to Dr Lau for member-checking, as done with Expert P ("Perfect") — his WhatsApp thread is active. To confirm: whether any audio was recorded (statement 6 permits it; researcher's notes presumed primary).